

ENGLISH FOR WORK / SEPTEMBER 2026

Pharmaceutical English



Learner workbook

Work with realistic cases, develop your own response, and use the answer section after you have tried the tasks.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Separate observations, assumptions, and conclusions.

02 Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

Make a request with a clear action, purpose, and realistic timing.

03 Clinical Trial Design, Protocols, GCP, and Operations

Clarify missing information before responding.

04 Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

Separate observations, assumptions, and conclusions.

05 Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

Give a handoff that the receiving person can confirm.

06 CMC, CGMP, Quality Events, Manufacturing, and Supply

Distinguish completed work from pending work and explain its impact.

07 Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

State a concern respectfully and propose a practical route forward.

08 Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

Explain a useful distinction in plain English.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop, or use the prompts at the end of this guide.

LESSON 01 / READ AND NOTICE

Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Separate observations, assumptions, and conclusions.

SITUATION

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

Indication - Disease, condition, or patient population for which a product is intended or approved.

MOA - Mechanism of action; how the product is understood to produce a biological effect.

TPP - Target product profile describing desired product attributes and evidence needs.

IND - Investigational new drug application allowing clinical investigation in humans in the United States.

3. Notice the language

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

The available evidence shows

This may indicate ..., but

We have not yet established whether

Improve this sentence: The pilot proves this will work everywhere.

LESSON 01 / PRACTICE AND PRODUCE

Match certainty to evidence

4. Check the language

1. Which sentence preserves uncertainty about an unconfirmed cause?
 - A. The change may have contributed to the delay.
 - B. The change must have caused the delay.
 - C. The change definitely caused the delay.
2. A result comes from a small pilot in one location. Which phrase accurately limits the claim?
 - A. In similar locations, we can now expect the same result ...
 - B. Across the service, the observed result was ...
 - C. In this pilot, the observed result was ...

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A laboratory signal is not patient benefit

Fictional development-slide review: a laboratory result is promising, but no clinical benefit has been established.

01 • Development lead: This slide says our candidate will benefit patients. What evidence supports that wording?

02 • Research scientist: We observed the intended response in the laboratory system, not in patients.

03 • Development lead: Could we describe that as confirmation of the proposed mechanism of action, or MOA?

04 • Research scientist: That is still too strong. The result supports our hypothesis under these experimental conditions.

05 • Development lead: Then how should we connect it to the target product profile, the TPP?

06 • Research scientist: Present the TPP as a development target, with clinical benefit listed as unestablished.

07 • Development lead: Should the indication remain on the slide as the proposed patient population?

08 • Research scientist: Yes, provided we label it proposed and avoid implying an approved use.

09 • Development lead: I'll replace the benefit claim and separate evidence from the development objective.

10 • Research scientist: I'll check that the laboratory conditions and remaining evidence gaps stay visible.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Two indications, one evidence package

Separate fictional portfolio discussion: candidate P7 has laboratory data for indication A; indication B has only a biological rationale.

01 • Portfolio manager: Can we put indications A and B at the same development stage?

02 • Translational scientist: Not on this evidence. P7 has laboratory data for A and a rationale for B.

03 • Portfolio manager: The proposed MOA is similar. Doesn't that support the same conclusion for both?

04 • Translational scientist: It supports asking the question, not assuming that the results will transfer.

05 • Portfolio manager: How do we explain the difference without making B sound irrelevant?

06 • Translational scientist: Call B an exploratory opportunity and identify the experiment needed to assess it.

07 • Portfolio manager: Would a separate TPP make that distinction clearer for the committee?

08 • Translational scientist: Possibly, but its targets must not appear to be measured product characteristics.

09 • Portfolio manager: I'll show observed evidence for A beside the unanswered questions for B.

10 • Translational scientist: Good. The committee can discuss priorities without confusing potential with demonstrated performance.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

Readiness for discussion versus readiness for filing

Separate fictional project check-in: an IND planning discussion is scheduled, but the supporting evidence inventory is incomplete.

01 • Project manager: The calendar says IND readiness. Can I report that the package is ready?

02 • Regulatory scientist: No. That is the meeting topic; our evidence inventory is still incomplete.

03 • Project manager: I understood the title to mean an investigational new drug submission was imminent.

04 • Regulatory scientist: We're reviewing gaps before proposing a filing schedule, not confirming a submission date.

05 • Project manager: Which part of the update can I state with confidence today?

06 • Regulatory scientist: The planning discussion is scheduled. The package assessment and timing remain open.

07 • Project manager: Should I include the proposed indication and TPP in that discussion?

08 • Regulatory scientist: Yes. They explain what we intend to develop and why particular evidence matters.

09 • Project manager: I'll correct the milestone wording and avoid treating a meeting as a completed assessment.

10 • Regulatory scientist: I'll bring the inventory and clearly distinguish available documents from expected ones.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Scenario 2 | An unmet-need claim without a source

In a fictional portfolio meeting, a slide says all existing options fail patients. The team has interviewed six clinicians, but no comparative evidence review is available. Discuss wording that preserves the development rationale without claiming universal failure.

Role A: Development strategist: explain what the interviews support and propose a qualified unmet-need statement.

Role B: Medical reviewer: challenge the universal wording and ask which evidence is still needed.

Possible opening: The six interviews identify concerns, but they don't establish that every existing option fails.

Success checks

- Distinguish reported concerns from a general clinical conclusion.
- Connect the proposed indication or TPP to an unanswered evidence question.
- Agree on a spoken claim that retains the limitation.

Add a complication: The presenter wants to keep the absolute claim because the speaking slot is only one minute.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 02 / READ AND NOTICE

Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

Make a request with a clear action, purpose, and realistic timing.

SITUATION

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

IND - Investigational new drug application allowing clinical investigation in humans in the United States.

NDA - New drug application requesting approval to market a drug.

BLA - Biologics license application requesting approval to market a biologic.

Complete response letter - FDA communication that an application is not ready for approval in its current form.

3. Notice the language

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Improve this sentence: Please revert soonest with the necessary.

LESSON 02 / PRACTICE AND PRODUCE

Ask for an actionable next step

4. Check the language

1. What does 'Please send it by Thursday' normally mean?
 - A. Send it on Thursday, but not earlier.
 - B. Keep sending it regularly until Thursday.
 - C. Send it no later than Thursday.
2. Which request gives the recipient enough detail to act?
 - A. Could you confirm that you received my earlier message?
 - B. Could you confirm the document version needed for the review?
 - C. Could you confirm that the review is important?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

A regulatory question the team can actually discuss

Fictional preparation for an agency meeting: the draft question asks whether the entire development plan is acceptable.

01 • Regulatory lead: Our question asks whether the whole plan is acceptable. What specific feedback do we need?

02 • Clinical lead: We need feedback on the proposed evidence supporting the next development stage.

03 • Regulatory lead: That still covers several topics. Which uncertainty affects our immediate planning decision?

04 • Clinical lead: Whether our proposed study population addresses the question described in the briefing document.

05 • Regulatory lead: Then let's ask about that population and provide our rationale, rather than request blanket approval.

06 • Clinical lead: Should we mention the IND in the opening sentence for context?

07 • Regulatory lead: Yes, but distinguish our current planning question from any future submission outcome.

08 • Clinical lead: I'll revise the question and identify the relevant briefing section this afternoon.

09 • Regulatory lead: Please send it for internal review before we finalize the meeting materials.

10 • Clinical lead: Understood. I'll request focused feedback without suggesting that a discussion guarantees acceptance.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Confirming which submission label belongs in the plan

Separate fictional planning meeting: a slide lists NDA/BLA as interchangeable placeholders; the regulatory strategy has not been confirmed.

01 • Program manager: Can you select NDA or BLA so our timeline looks complete?

02 • Regulatory strategist: I can review the terminology, but I shouldn't choose a route for appearance.

03 • Program manager: What information do you need from the technical team to assess it?

04 • Regulatory strategist: Please provide the current product description and the assumptions behind the proposed submission strategy.

05 • Program manager: Could we call both labels equivalent until that review is finished?

06 • Regulatory strategist: No. A new drug application and a biologics license application are distinct submission terms.

07 • Program manager: I'll mark the route as under review. Can you assess the description before Thursday's planning session?

08 • Regulatory strategist: I can give an initial assessment then, with unresolved questions clearly identified.

09 • Program manager: That will let us discuss dependencies without presenting an invented regulatory conclusion.

10 • Regulatory strategist: Agreed. I'll also specify what further specialist review is needed before the label is finalized.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Assigning questions after a response letter

Separate fictional internal exercise: a complete response letter lists three issues, and the team has not yet assigned reviewers.

01 • Regulatory coordinator: We have three issues in the complete response letter and no assigned reviewers yet.

02 • Program lead: Can we tell the board that resolving those issues guarantees approval?

03 • Regulatory coordinator: No. We can describe our response planning, not promise the outcome of another review.

04 • Program lead: What action do you need from me before tomorrow's internal meeting?

05 • Regulatory coordinator: Please nominate an accountable reviewer for each issue and confirm their availability.

06 • Program lead: Should those reviewers commit to completion dates immediately, before assessing the requested work?

07 • Regulatory coordinator: Ask for initial scope estimates first. Some dependencies may still be unknown.

08 • Program lead: I'll request owners today and ask them to identify unanswered questions tomorrow.

09 • Regulatory coordinator: I'll keep each response item linked to the relevant wording in the letter.

10 • Program lead: Then our update can distinguish assigned work, estimated timing, and decisions we cannot predict.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Scenario 2 | An unanswered meeting-minutes discrepancy

In a fictional regulatory follow-up, internal notes say a proposed analysis was discussed; the draft summary says it was agreed. No final agency minutes are available. Conduct a call to request a precise wording review before circulation tomorrow.

Role A: Regulatory coordinator: identify the discrepancy and request review of the specific sentence.

Role B: Clinical representative: explain the internal recollection without claiming confirmed agreement.

Possible opening: Our notes record discussion, but the summary says agreement; could you review that distinction today?

Success checks

- Identify the exact wording needing review.
- Distinguish internal recollection from confirmed agency feedback.
- Agree an owner and a realistic response time without inventing approval.

Add a complication: A project sponsor asks you to circulate the stronger version immediately.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 03 / READ AND NOTICE

Clinical Trial Design, Protocols, GCP, and Operations

Clarify missing information before responding.

SITUATION

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

Protocol - Document describing study objectives, design, population, assessments, endpoints, safety, and analysis.

Randomization - Assignment to treatment groups by chance to reduce bias.

Informed consent - Process and documentation showing that participants understand key trial information before participation.

Protocol deviation - Departure from the approved protocol or study procedures.

3. Notice the language

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Improve this sentence: Could you explain what does this mean?

LESSON 03 / PRACTICE AND PRODUCE

Ask a precise question

4. Check the language

1. Which indirect question has the usual word order?
 - A. Could you confirm when starts the review?
 - B. Could you confirm when the review starts?
 - C. Could you confirm when does the review start?
2. Someone says the report is 'ready.' Which question best clarifies its approval status?
 - A. Has the report been approved, or is it ready for review?
 - B. Can you send the report to me this afternoon?
 - C. Who will present the report at the meeting?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Which visit window controls?

Fictional trial coordination call: two documents describe different visit windows, and the controlling version is unconfirmed.

01 • Site coordinator: The visit guide and protocol give different windows. Which one should I communicate?

02 • Clinical monitor: Before answering, could you confirm the version identifiers on both documents?

03 • Site coordinator: I can read them back, but I haven't checked which version is controlling.

04 • Clinical monitor: Then let's verify that through document control instead of choosing the more convenient window.

05 • Site coordinator: The scheduling team wants an answer now. Can I call the difference a protocol deviation?

06 • Clinical monitor: Not from the discrepancy alone. First establish the applicable requirement and what actually happened.

07 • Site coordinator: I'll explain that the scheduling instruction is awaiting confirmation, without giving a replacement window.

08 • Clinical monitor: Please send the conflicting passages through the trial's authorized document channel.

09 • Site coordinator: Will you take ownership of checking the controlling version and returning a clear answer?

10 • Clinical monitor: Yes. I'll confirm the requirement with the responsible team and close the communication loop.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

A consent discussion is not a completed consent record

Separate fictional training case: a tracker says consent discussed, while a meeting agenda says ready for randomization.

01 • Trial coordinator: Our agenda says ready for randomization, but the tracker only says consent discussed.

02 • Study nurse: That wording doesn't tell us whether the required informed consent process is complete.

03 • Trial coordinator: Could the two entries simply mean the same thing in different words?

04 • Study nurse: We need confirmation, not an assumption. Which authorized record supports the readiness status?

05 • Trial coordinator: I haven't checked it, and I don't have a verified completion entry.

06 • Study nurse: Then ask the responsible site clinician to confirm the status under the protocol.

07 • Trial coordinator: Should I change the agenda to consent status awaiting verification for now?

08 • Study nurse: Yes. That describes our knowledge without implying either completion or a confirmed failure.

09 • Trial coordinator: I'll also ask who updates the tracker once the status is verified.

10 • Study nurse: Good. We'll use the confirmed record for the next communication, not infer readiness from a heading.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

Allocation wording in a recruitment briefing

Separate fictional recruitment rehearsal: a draft describes randomization as the coordinator choosing the most suitable study group.

01 • Recruitment coordinator: The briefing says I'll choose the most suitable group. Is that our randomization process?

02 • Trial educator: That wording sounds like personal selection. What does the approved protocol explanation say?

03 • Recruitment coordinator: It describes assignment through the study's randomization process, without a coordinator choosing.

04 • Trial educator: Then we should use that explanation and remove the suggestion of individual selection.

05 • Recruitment coordinator: Could I explain the exact allocation ratio to make the process clearer?

06 • Trial educator: Only after checking the approved information. We haven't established that ratio in this rehearsal.

07 • Recruitment coordinator: I'll ask which participant-facing wording is approved before the next practice session.

08 • Trial educator: Also distinguish explaining assignment from making promises about which group someone will enter.

09 • Recruitment coordinator: Understood. I'll clarify the process and refer questions beyond the materials to the study team.

10 • Trial educator: Let's rehearse that corrected explanation once the source wording has been confirmed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Clinical Trial Design, Protocols, GCP, and Operations

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Scenario 2 | Remote or on-site visit?

A fictional trial calendar labels a follow-up remote, while a training checklist says on-site. The participant has not been contacted, and the applicable protocol version is unknown. Discuss how to obtain a reliable scheduling instruction.

Role A: Site scheduler: ask focused questions about the visit type and controlling document.

Role B: Trial operations specialist: acknowledge what is unknown and take responsibility for verification.

Possible opening: Before I contact the participant, could we confirm whether this visit is remote or on-site?

Success checks

- Do not treat the most recent edit as proof of authority.
- Name the document and visit details requiring confirmation.
- Agree who returns the verified instruction before participant contact.

Add a complication: A colleague suggests using the calendar because it was edited most recently.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 04 / READ AND NOTICE

Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

Separate observations, assumptions, and conclusions.

SITUATION

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

Primary endpoint - Main outcome measure used to evaluate the primary objective.

Secondary endpoint - Additional outcome measure supporting other objectives.

Estimand - Precise description of the treatment effect being estimated.

Confidence interval - An interval produced by a method with a stated long-run coverage rate under its assumptions.

3. Notice the language

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

The available evidence shows

This may indicate ..., but

We have not yet established whether

Improve this sentence: The pilot proves this will work everywhere.

LESSON 04 / PRACTICE AND PRODUCE

Match certainty to evidence

4. Check the language

1. Which sentence preserves uncertainty about an unconfirmed cause?
 - A. The change may have contributed to the delay.
 - B. The change must have caused the delay.
 - C. The change definitely caused the delay.
2. A result comes from a small pilot in one location. Which phrase accurately limits the claim?
 - A. In similar locations, we can now expect the same result ...
 - B. Across the service, the observed result was ...
 - C. In this pilot, the observed result was ...

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

The highlighted endpoint is not the primary result

Fictional trial-slide review: a secondary endpoint is highlighted, while the primary endpoint result needs explanation.

01 • Medical writer: The headline highlights the secondary endpoint. Will listeners think that was the main study question?

02 • Biostatistician: They may. We need the primary endpoint result and its interpretation visible first.

03 • Medical writer: Can I simply describe the highlighted result as a successful trial outcome?

04 • Biostatistician: That would exceed what this slide establishes. The endpoint hierarchy affects the interpretation.

05 • Medical writer: What should the audience hear before we discuss the additional result?

06 • Biostatistician: Which endpoint was primary, what was observed, and what uncertainty remains in that estimate.

07 • Medical writer: Should the confidence interval stay beside the estimate rather than in a footnote?

08 • Biostatistician: Yes. Keep uncertainty accessible, and have the full statistical interpretation reviewed.

09 • Medical writer: I'll reorganize the slide without inventing a conclusion about overall study success.

10 • Biostatistician: I'll check the endpoint descriptions and explain any limitations on the secondary analysis.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

A precise number with an imprecise estimate

Separate fictional analysis rehearsal: an estimated difference is four units with a confidence interval from minus two to ten; clinical importance has not been assessed.

01 • Clinical scientist: The difference is four units. Can we call that a meaningful improvement?

02 • Biostatistician: Not from the point estimate alone. The confidence interval runs from minus two to ten.

03 • Clinical scientist: So should I say there is definitely no difference between the groups?

04 • Biostatistician: No. That also overstates the result. Explain the estimate and its uncertainty.

05 • Clinical scientist: What about calling four units clinically important for this outcome measure?

06 • Biostatistician: We haven't assessed clinical importance here. Don't supply a threshold we haven't established.

07 • Clinical scientist: I'll state the numbers, then separate statistical uncertainty from the clinical interpretation.

08 • Biostatistician: Please also identify the outcome measure and analysis population in the supporting material.

09 • Clinical scientist: Those details need verification before I present them, so I'll request the analysis source.

10 • Biostatistician: Good. We can give an accurate numerical summary without turning it into an unsupported benefit claim.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

Two analyses answer different questions

Separate fictional methods discussion: two trial analyses handle treatment discontinuation differently; neither result is supplied in the exercise.

01 • Clinical lead: Why do we need two analysis descriptions if they concern the same treatment?

02 • Statistician: They handle treatment discontinuation differently, so their estimands may address different treatment-effect questions.

03 • Clinical lead: Is an estimand just another name for the statistical calculation we run?

04 • Statistician: No. It specifies the effect we want to estimate; the calculation is a separate matter.

05 • Clinical lead: Could we select whichever analysis eventually produces the more attractive estimate?

06 • Statistician: We should explain the prespecified question and analysis plan, not choose a story after seeing results.

07 • Clinical lead: What can I tell the medical team while the numerical results are unavailable?

08 • Statistician: Describe the differing questions and the handling of discontinuation, without claiming which result is stronger.

09 • Clinical lead: I'll ask you to review that explanation before the data-readout rehearsal.

10 • Statistician: Agreed. We'll make the interpretive boundary clear before discussing any eventual endpoint estimates.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Scenario 2 | A subgroup headline without context

A fictional trial summary highlights one subgroup from an exploratory analysis. The overall primary endpoint result is available elsewhere but is not included in the summary. No interaction analysis is supplied. Discuss a responsible spoken introduction.

Role A: Medical communications lead: qualify the subgroup statement and request the missing context.

Role B: Study statistician: explain why the subgroup alone cannot establish a different treatment effect.

Possible opening: This is an exploratory subgroup finding, and we need the overall primary result beside it.

Success checks

- Identify the exploratory status.
- Avoid claiming a subgroup difference that has not been established.
- Agree which primary-result and uncertainty information must accompany the statement.

Add a complication: The presenter asks you to describe the subgroup as the patients who definitely benefit.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 05 / READ AND NOTICE

Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

Give a handoff that the receiving person can confirm.

SITUATION

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

AE - Adverse event; unfavorable medical occurrence after product use, regardless of causality.

SAE - Serious adverse event meeting criteria such as death, life-threatening event, hospitalization, disability, or birth defect.

Signal detection - Process for identifying information that may suggest a new or changed risk.

Risk-benefit - Integrated assessment of benefits, risks, uncertainty, disease severity, and alternatives.

3. Notice the language

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Improve this sentence: Someone should follow up on this.

LESSON 05 / PRACTICE AND PRODUCE

Transfer information and responsibility

4. Check the language

1. Which sentence checks that responsibility has been accepted?
 - A. I mentioned the check in my earlier email.
 - B. The check appears somewhere in the notes.
 - C. Please confirm who will complete the outstanding check.
2. Which response best checks a critical handoff detail?
 - A. I saw the message, so no clarification is needed.
 - B. So the result is pending and your team will follow up; is that correct?
 - C. I assume all results are final, so we can finish.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Passing an incomplete event report

Fictional safety-reporting exercise: a possible adverse event has been reported with incomplete details; the team has a defined reporting process.

01 • Medical information specialist: I received a possible adverse event report, but several details are missing.

02 • Safety associate: Please transfer the available information through our safety channel and identify what remains unknown.

03 • Medical information specialist: Should I wait until I can establish whether the product caused it?

04 • Safety associate: No. Our handoff should preserve the report without making a causality judgment.

05 • Medical information specialist: I also don't know whether the reported event meets the seriousness criteria.

06 • Safety associate: State that clearly. The responsible safety reviewer will assess the report under our process.

07 • Medical information specialist: I'll pass the reporter's wording and distinguish it from my own questions.

08 • Safety associate: Please include the available contact route in the restricted record, not general team chat.

09 • Medical information specialist: Can you confirm receipt and take responsibility for the next follow-up step?

10 • Safety associate: Yes. I'll acknowledge the handoff and flag the missing details for the assigned reviewer.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

Severe wording does not settle seriousness

Separate fictional safety intake: a caller described an event as severe; hospitalization and other seriousness information are unknown.

01 • Intake coordinator: The caller said severe. I've prepared a handoff, but the seriousness details are unknown.

02 • Pharmacovigilance reviewer: Keep the caller's description. Don't silently change severe to serious in the record.

03 • Intake coordinator: I thought those words were interchangeable when describing an SAE, a serious adverse event.

04 • Pharmacovigilance reviewer: They address different concepts. We need the relevant case information and an authorized assessment.

05 • Intake coordinator: Should I classify it as non-serious because hospitalization wasn't mentioned during the call?

06 • Pharmacovigilance reviewer: No. Missing information is not evidence that a seriousness criterion is absent.

07 • Intake coordinator: I'll mark the details unknown and hand over the questions already asked.

08 • Pharmacovigilance reviewer: Thank you. I'll arrange follow-up through the case process and preserve the original wording.

09 • Intake coordinator: Please confirm that you now own follow-up, so the reporter doesn't receive duplicate calls.

10 • Pharmacovigilance reviewer: Confirmed. I'll coordinate the next contact and document the assessment when information is available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

A cluster passed to signal review

Separate fictional surveillance meeting: three reports describe a similar event, but duplicate status and exposure information have not been checked.

01 • Safety analyst: Three reports describe similar events. I'm handing them to you for signal review.

02 • Signal lead: Have you confirmed that they represent three separate cases rather than duplicate reports?

03 • Safety analyst: Not yet. Exposure information is also missing, so I haven't calculated an event rate.

04 • Signal lead: Good. Please preserve those limitations instead of presenting a confirmed increase in risk.

05 • Safety analyst: Can I say signal detection has established a causal association with the product?

06 • Signal lead: No. These reports prompt assessment; they don't establish causality or a risk-benefit conclusion.

07 • Safety analyst: I'll link the case references and summarize the shared wording without adding a diagnosis.

08 • Signal lead: I'll assign duplicate review and confirm what information is needed for further assessment.

09 • Safety analyst: Who should receive questions if another similar report arrives during your review?

10 • Signal lead: Route it through the existing safety process and reference this review so we can reconcile the records.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Scenario 2 | A reporter corrects the event date

A fictional safety case is already assigned to reviewer Mina. The reporter now says the recorded event date was a transcription error but cannot recall the correct date. Practice handing over this correction without deleting the history or inventing a replacement.

Role A: Medical information specialist: transfer the correction and identify the remaining uncertainty.

Role B: Safety reviewer Mina: confirm receipt and explain who coordinates further clarification.

Possible opening: The reporter has withdrawn the recorded date, but a replacement date is still unknown.

Success checks

- Separate the contact date from the unknown event date.
- Preserve the original report and correction through the case process.
- Confirm the receiving owner and next follow-up responsibility.

Add a complication: A colleague suggests using the first contact date as the event date to avoid an empty field.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 06 / READ AND NOTICE

CMC, CGMP, Quality Events, Manufacturing, and Supply

Distinguish completed work from pending work and explain its impact.

SITUATION

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

CMC - Chemistry, manufacturing, and controls information supporting product quality.

Batch record - Documented record of manufacturing steps, controls, and results for a batch.

OOS - Out of specification result requiring investigation.

CAPA - Corrective and preventive action addressing root cause and recurrence prevention.

3. Notice the language

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Improve this sentence: Everything is progressing well.

LESSON 06 / PRACTICE AND PRODUCE

Give a useful status update

4. Check the language

1. Which sentence says a review is finished?
 - A. The team plans to complete the review.
 - B. The team has completed the review.
 - C. The team is completing the review.
2. Which update distinguishes a plan from a confirmed fact?
 - A. Delivery is planned for Friday; the carrier has not confirmed it.
 - B. Delivery is confirmed for Friday because we planned it.
 - C. The delivery plan proves that arrival will be Friday.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

A batch review with missing documentation

Fictional quality meeting: documentation is missing from a batch record, and a manager asks whether review is complete.

01 • Production manager: Can I describe the batch as fully reviewed in today's supply update?

02 • Quality reviewer: Not yet. The batch record review identified missing documentation that remains unresolved.

03 • Production manager: The manufacturing steps are complete. Isn't fully reviewed a reasonable shorthand for that?

04 • Quality reviewer: Those are different statuses. Completed production doesn't establish completed quality review or authorized disposition.

05 • Production manager: What wording will explain the situation without implying a confirmed product defect?

06 • Quality reviewer: Say production is complete and document review remains open because supporting information is missing.

07 • Production manager: Should I call this OOS in the update to emphasize its importance?

08 • Quality reviewer: No. An out-of-specification result is not established by the documentation gap described here.

09 • Production manager: I'll keep the supply forecast conditional and request the next review-status checkpoint.

10 • Quality reviewer: I'll provide that checkpoint and identify what remains outstanding without predicting the disposition decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

An OOS investigation is not a release decision

Separate fictional quality exercise: one test result is recorded as OOS, the investigation is open, and no disposition is authorized.

01 • Supply planner: The investigation has started. Does that mean the batch will be available this week?

02 • Quality investigator: No. One result is out of specification, and the investigation and disposition remain open.

03 • Supply planner: Can I remove the delay from the supply dashboard while you repeat the test?

04 • Quality investigator: We haven't agreed a repeat test here. Don't assume a particular investigation step or outcome.

05 • Supply planner: What completed action can I report to the commercial planning team today?

06 • Quality investigator: The OOS result has been logged and assigned for investigation. Availability is not confirmed.

07 • Supply planner: Would describing it as a CMC quality issue help explain the dependency?

08 • Quality investigator: Yes, with the specific status. Broad terminology shouldn't hide the unresolved batch decision.

09 • Supply planner: I'll show an unconfirmed supply date and keep any alternative plan separate.

10 • Quality investigator: I'll update you when the authorized review changes status, not merely when an activity is scheduled.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

CAPA implementation versus demonstrated effectiveness

Separate fictional CAPA review: a revised batch-record checklist is in use, but the planned effectiveness check has not occurred.

01 • Quality manager: The new checklist is in use. Can we close the corrective and preventive action?

02 • CAPA owner: Implementation is complete, but the planned effectiveness check hasn't happened yet.

03 • Quality manager: Staff say the checklist is clearer. Doesn't that demonstrate that the issue is resolved?

04 • CAPA owner: It is useful feedback, but it doesn't replace the agreed evidence for effectiveness.

05 • Quality manager: How should the progress report distinguish those two stages of the work?

06 • CAPA owner: List the revised checklist as implemented and the effectiveness assessment as pending.

07 • Quality manager: Could we describe the batch-record problem as eliminated across every production line?

08 • CAPA owner: Not on this evidence. Our change and planned assessment have a defined scope.

09 • Quality manager: I'll keep that scope visible and ask when the assessment results can be reviewed.

10 • CAPA owner: I'll confirm the assessment schedule and report findings without treating completion of training as proof of prevention.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | CMC, CGMP, Quality Events, Manufacturing, and Supply

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Second round: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Model for scenario 1: The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Scenario 2 | A supplier certificate arrives unread

In a fictional CMC supply meeting, the missing supplier certificate has arrived for lot L8. Quality has not reviewed it, and disposition remains pending. Discuss what changed and what did not change in the lot's status.

Role A: Materials coordinator: report receipt accurately without implying release.

Role B: Quality reviewer: clarify the remaining document check and take responsibility for the next update.

Possible opening: The certificate for L8 has arrived, but receipt is not the same as completed quality review.

Success checks

- Distinguish document receipt from review and disposition.
- Avoid assuming the certificate is acceptable before assessment.
- Agree who communicates the next verified status.

Add a complication: A planner asks to change the availability indicator to confirmed immediately.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 07 / READ AND NOTICE

Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

State a concern respectfully and propose a practical route forward.

SITUATION

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

Prescribing information - Approved product information describing indication, dosing, warnings, adverse reactions, and other use information.

Fair balance - Balanced communication of benefits and risks so the message is not misleading.

MLR - Medical, legal, and regulatory review of materials.

Scientific exchange - Non-promotional scientific communication, usually handled within medical affairs boundaries.

3. Notice the language

Acknowledge the goal without pretending agreement. Name the specific problem and its implication, then ask a focused question or propose a next step. Be direct when a safety or ethical boundary requires it.

I understand the goal. My concern is

That statement goes beyond

Could we ... before confirming ...?

Improve this sentence: That idea is ridiculous.

LESSON 07 / PRACTICE AND PRODUCE

Disagree with a useful reason

4. Check the language

1. Which response identifies a specific assumption and evidence against it?
 - A. The proposal assumes the review is complete; the record shows it is still open.
 - B. The proposal appears optimistic, and I would prefer a slower approach.
 - C. I would like another discussion before we proceed with the proposal.
2. Which follow-up makes disagreement actionable?
 - A. Could we confirm the plan and discuss the missing approval later?
 - B. Could we ask for general feedback before sending the next update?
 - C. Could we resolve the missing approval before confirming the plan?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

A stronger adjective than the reviewed material

Fictional promotional review: a draft slide makes a stronger benefit claim than the supplied approved material.

01 • Brand manager: The stronger headline is clearer. Can we keep it for tomorrow's presentation?

02 • Medical reviewer: My concern is that it goes beyond the benefit language in the supplied approved material.

03 • Brand manager: Could a reference to the prescribing information at the bottom solve that problem?

04 • Medical reviewer: A reference doesn't make an unsupported headline accurate. We need the claim itself reviewed.

05 • Brand manager: What can we do within the remaining preparation time without inventing new wording?

06 • Medical reviewer: Use the supplied language as the starting point and route the revision through MLR.

07 • Brand manager: By MLR, you mean medical, legal, and regulatory review, not just your personal approval?

08 • Medical reviewer: Correct. Also preserve the relevant risk context and fair balance for the proposed material.

09 • Brand manager: I'll replace the stronger claim and keep the presentation status pending that review.

10 • Medical reviewer: Thank you. I'll identify the precise wording concern so the reviewers can address it efficiently.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

Scientific exchange is not a label for any slide

Separate fictional conference preparation: a commercial deck contains unreviewed exploratory findings; its owner proposes calling the deck scientific exchange.

01 • Commercial lead: Could we call this scientific exchange and keep the exploratory findings in the deck?

02 • Medical affairs lead: Changing the label doesn't settle the purpose, audience, or appropriate review of the content.

03 • Commercial lead: The findings are interesting. Why can't the presenter simply describe them as promising?

04 • Medical affairs lead: We need to assess the actual claims and limitations, not rely on a reassuring adjective.

05 • Commercial lead: What practical alternative preserves the audience's opportunity to ask scientific questions?

06 • Medical affairs lead: Keep this presentation within its reviewed scope and refer scientific inquiries through the appropriate medical channel.

07 • Commercial lead: Should I send you the question and the study source separately from the deck?

08 • Medical affairs lead: Yes. We'll assess an appropriate response without assuming this commercial material is the right vehicle.

09 • Commercial lead: I'll remove the unreviewed section from the proposed presentation and flag the inquiry.

10 • Medical affairs lead: I'll coordinate the relevant review and avoid promising that any particular content will be approved.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

A risk panel lost in a smaller format

Separate fictional design review: resizing a promotional panel has made the supplied risk text unreadable; no replacement layout is approved.

01 • Design lead: The compact version fits the screen, but the risk panel is now very small.

02 • Compliance reviewer: That creates a fair-balance concern. The benefit headline is prominent while the risk text is unreadable.

03 • Design lead: Could we keep the headline and move all risk context to a later slide?

04 • Compliance reviewer: I can't approve that shortcut. The revised presentation needs review in its actual format.

05 • Design lead: What change can I prepare that gives the reviewers something workable to assess?

06 • Compliance reviewer: Try a larger layout using the supplied content, with both benefit and risk information legible.

07 • Design lead: Would shortening the prescribing-information extract require a fresh content review as well?

08 • Compliance reviewer: Yes, route any proposed wording change through the relevant review rather than editing it independently.

09 • Design lead: I'll prepare a readable version and mark it as a draft for MLR.

10 • Compliance reviewer: Good. We'll review the complete presentation, including how the information appears to the audience.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You want to move quickly and initially overlook a relevant condition. Ask why it matters, then consider a practical route forward that respects the real constraint.

Second round: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

Model for scenario 1: The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Scenario 2 | A testimonial asks for a stronger promise

A fictional promotional video draft includes a speaker saying the product always works. The supplied approved material contains no universal benefit claim. Discuss a revision request before recording; no replacement statement has been reviewed.

Role A: Production coordinator: explain the recording constraint and ask for usable reviewed wording.

Role B: Medical reviewer: challenge the absolute claim and propose the review route.

Possible opening: The word always promises more than our supplied material supports, so this line needs review before recording.

Success checks

- Identify the specific unsupported promise.
- Avoid treating a testimonial label as an automatic exemption.
- Agree to obtain reviewed wording without inventing a new benefit claim.

Add a complication: The speaker says the sentence is a personal opinion and therefore needs no review.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 08 / READ AND NOTICE

Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

Explain a useful distinction in plain English.

SITUATION

A market-access presentation combines trial findings and observational data without identifying the different study designs.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

HEOR - Health economics and outcomes research.

Formulary - List of medicines covered or preferred by a payer or health system.

RWD - Real-world data routinely collected from health care or patient sources.

RWE - Clinical evidence about usage, benefits, or risks derived from RWD analysis.

3. Notice the language

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Improve this sentence: We need to operationalize the implementation.

LESSON 08 / PRACTICE AND PRODUCE

Make a complex point clear

4. Check the language

1. Which sentence explains 'backlog' rather than repeating it?
 - A. The backlog is the work completed during the last period.
 - B. The backlog is the maximum work the team can finish in a period.
 - C. The backlog is the work still waiting to be completed.
2. Which follow-up best checks whether your explanation was clear?
 - A. Was the explanation detailed enough for you?
 - B. How would you describe the next step in your own words?
 - C. Would you like me to repeat the explanation?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

Trial findings beside observational evidence

Fictional market-access rehearsal: trial findings and observational data appear together without study-design labels.

01 • Market access manager: The presentation combines these results. Why do we need separate study-design labels?

02 • HEOR scientist: Trial findings and observational analyses have different conditions and limitations.

03 • Market access manager: Does that mean the observational information has no place here?

04 • HEOR scientist: No. Health economics and outcomes research can use it, with its source and limits explained.

05 • Market access manager: How do I distinguish real-world data, RWD, from real-world evidence, RWE?

06 • HEOR scientist: RWD are routinely collected healthcare and patient data. RWE is clinical evidence about use, benefits, or risks derived from analyzing those data.

07 • Market access manager: Can I present both sources as equally establishing that the product caused the outcome?

08 • HEOR scientist: Not without examining the designs. Putting results together doesn't make their causal interpretations equivalent.

09 • Market access manager: I'll label each source and separate findings from any formulary recommendation.

10 • HEOR scientist: I'll check that our explanation of the limitations matches the actual analyses.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A formulary decision is not a universal access promise

Separate fictional launch meeting: one organization's formulary listing is confirmed, while patient-level access conditions have not been reviewed.

01 • Launch coordinator: We have a formulary listing. Can the field update say access is now unrestricted?

02 • Access specialist: No. The listing is confirmed, but we haven't reviewed the patient-level access conditions.

03 • Launch coordinator: I thought formulary meant everyone covered by that organization could obtain the product immediately.

04 • Access specialist: It identifies inclusion on a medicine list; the actual terms need separate checking.

05 • Launch coordinator: What distinction should the update make for colleagues planning launch activities this week?

06 • Access specialist: State the confirmed listing and flag that operational access details remain under review.

07 • Launch coordinator: Could HEOR results tell us those administrative conditions without obtaining the listing documents?

08 • Access specialist: No. Outcomes evidence and access requirements are different information sources for different questions.

09 • Launch coordinator: I'll request the relevant documents and remove the unrestricted-access phrase from the draft.

10 • Access specialist: I'll check the conditions and provide a scoped explanation rather than a promise about individual coverage.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

Database size versus a usable comparison

Separate fictional RWE planning discussion: access to a database of 50,000 routinely collected patient records is confirmed, but completeness of the proposed outcome and comparator variables is unknown.

01 • Business development lead: Fifty thousand records sounds persuasive. Can we say the evidence package is already strong?

02 • RWE analyst: We have a large data source, but outcome and comparator completeness are still unknown.

03 • Business development lead: Does the number of records compensate for those missing details in the analysis?

04 • RWE analyst: Not automatically. Record count doesn't tell us whether the data can answer the proposed question.

05 • Business development lead: What should our lifecycle planning discussion distinguish before we promise a deliverable?

06 • RWE analyst: Data availability from analytical fitness. We need to assess the variables, population, and proposed comparison.

07 • Business development lead: Could we call this RWD access secured, with the RWE feasibility assessment pending?

08 • RWE analyst: Yes. Access is confirmed, but we shouldn't imply the analysis has already produced clinical evidence.

09 • Business development lead: I'll report the confirmed access separately from the pending assessment so the distinction stays clear.

10 • RWE analyst: I'll report feasibility findings before recommending an outcome claim or a timetable for the evidence package.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

A market-access presentation combines trial findings and observational data without identifying the different study designs.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Scenario 2 | Launch activity versus access outcome

A fictional launch dashboard records twelve payer meetings but no formulary decisions yet. A slide calls the meetings twelve access wins. Discuss how to explain the activity and the unconfirmed outcome in a short leadership conversation.

Role A: Launch operations lead: explain what the twelve meetings measure.

Role B: Market-access director: ask which evidence would support a confirmed access outcome.

Possible opening: We completed twelve payer meetings; that is an activity count, not twelve confirmed formulary decisions.

Success checks

- Separate completed meetings from formulary outcomes.
- Do not invent payer decisions or coverage conditions.
- Agree a concise label and an owner for tracking decisions.

Add a complication: A colleague wants to keep wins because the dashboard has no space for caveats.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

Answer workshop

Try the tasks first. The model is one acceptable response to scenario 1, not a script to memorize. Assess your spoken version using the four criteria at the end.

01 | Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Editing example: The pilot worked in the tested setting; performance elsewhere remains untested.

The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Check 1: A - The change may have contributed to the delay.

A: 'May have contributed' presents a possibility without claiming the cause is established.

B: 'Must have' expresses a strong inference that the evidence may not support.

C: 'Definitely' presents the cause as certain.

Check 2: C - In this pilot, the observed result was ...

A: Similarity alone does not establish that the result will transfer.

B: A pilot in one location does not establish a service-wide observation.

C: This locates the finding in the setting actually studied.

Model response: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

02 | Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

Editing example: Could you send the missing document and confirm when it will be available?

The revision names the document and requested response. It avoids expressions that may be unfamiliar or ambiguous for an international audience.

Check 1: C - Send it no later than Thursday.

A: "By Thursday" permits earlier delivery; it is not restricted to that day.

B: This interprets a deadline as a continuing activity, which would use "until".

C: 'By' sets the latest point, while 'after' refers to a later time.

Check 2: B - Could you confirm the document version needed for the review?

A: This checks receipt, not which version is required for the review.

B: The requested action and information are explicit.

C: This checks importance rather than identifying the needed document version.

Model response: Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

03 | Clinical Trial Design, Protocols, GCP, and Operations

Editing example: Could you explain what this means?

After an introductory phrase such as 'Could you explain', the embedded question uses statement word order: "what this means".

Check 1: B - Could you confirm when the review starts?

A: Place the subject 'the review' before 'starts'.

B: The embedded question uses subject then verb: "the review starts".

C: Remove 'does' and use statement word order after 'confirm'.

Check 2: A - Has the report been approved, or is it ready for review?

A: This question distinguishes two meanings of 'ready' that affect the next action.

B: This asks about delivery timing, not whether approval has happened.

C: This asks about the presenter, not the report's approval status.

Model response: The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

04 | Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

Editing example: The pilot worked in the tested setting; performance elsewhere remains untested.

The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Check 1: A - The change may have contributed to the delay.

A: 'May have contributed' presents a possibility without claiming the cause is established.

B: 'Must have' expresses a strong inference that the evidence may not support.

C: 'Definitely' presents the cause as certain.

Check 2: C - In this pilot, the observed result was ...

A: Similarity alone does not establish that the result will transfer.

B: A pilot in one location does not establish a service-wide observation.

C: This locates the finding in the setting actually studied.

Model response: Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

05 | Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

Editing example: Please confirm which team has accepted the follow-up task.

The revised sentence asks for an identifiable receiving team instead of leaving responsibility with an unspecified 'someone'.

Check 1: C - Please confirm who will complete the outstanding check.

A: Mentioning a task does not show that anyone accepted it.

B: A written record alone does not identify who will act.

C: It asks for an identifiable person or team responsible for the task.

Check 2: B - So the result is pending and your team will follow up; is that correct?

A: Receiving a message is not the same as confirming its meaning.

B: This repeats the status and responsibility and invites correction.

C: It changes pending information into a final result.

Model response: I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

06 | CMC, CGMP, Quality Events, Manufacturing, and Supply

Editing example: Three checks are complete; two remain open and are being reviewed.

The revision gives observable progress. A positive label alone does not tell the reader what is complete or still needed.

Check 1: B - The team has completed the review.

A: 'Plans to complete' describes an intention, not completion.

B: 'Has completed' states that the action is finished.

C: 'Is completing' presents the work as in progress.

Check 2: A - Delivery is planned for Friday; the carrier has not confirmed it.

A: This explicitly labels the date as planned and identifies the missing confirmation.

B: A plan does not establish carrier confirmation.

C: A plan cannot prove a future arrival time.

Model response: The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

07 | Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

Editing example: My concern is that the proposal assumes capacity we have not confirmed.

The revised sentence identifies a testable concern about the proposal. It leaves room for a constructive response.

Check 1: A - The proposal assumes the review is complete; the record shows it is still open.

A: It connects the concern to a specific assumption and record.

B: This expresses a preference, but does not identify the assumption or the record that contradicts it.

C: This requests discussion without stating the specific assumption or evidence.

Check 2: C - Could we resolve the missing approval before confirming the plan?

A: This confirms the plan before resolving the stated approval condition.

B: General feedback does not directly address the missing approval.

C: It names the unresolved condition and a practical next step.

Model response: The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

08 | Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

Editing example: We need to put the plan into practice.

The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Check 1: C - The backlog is the work still waiting to be completed.

A: Completed work is different from work still waiting.

B: This describes capacity, not the work waiting to be completed.

C: This gives the meaning in familiar words.

Check 2: B - How would you describe the next step in your own words?

A: This invites an opinion on detail, not a demonstration of understanding.

B: An open request lets the listener show their understanding.

C: Offering repetition may help, but it does not show how the listener understood the next step.

Model response: Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-06. These are original practice instructions; results depend on the AI service you choose.

Practice grammar in context

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Pharmaceutical English

Professional audience: clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

Original fictional case: A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

Language workshop: Match certainty to evidence

Communication goal: Separate observations, assumptions, and conclusions.

Language guidance: Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Useful frames (complete the gaps with case facts): The available evidence shows ... / This may indicate ..., but ... / We have not yet established whether ...

Vocabulary: Indication: Disease, condition, or patient population for which a product is intended or approved. / MOA: Mechanism of action; how the product is understood to produce a biological effect. / TPP: Target product profile describing desired product attributes and evidence needs. / IND: Investigational new drug application allowing clinical investigation in humans in the United States.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for comparison AFTER my attempt, not a response to give me first: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

END REFERENCE

TASK: Grammar that changes the meaning

1. For a lesson, use its language workshop as the focus. For a dialogue, quote one actual sentence and select one useful grammar feature from it, such as question word order, tense, modality, conditionals or clause linking. Explain in no more than 90 words how this feature helps the speakers do their work. Keep the grammar target narrow.
2. Give two short contrasting sentences using this workplace context. Explain the difference in time, certainty, condition or politeness. Label invented examples and preserve the distinction between possible, planned and confirmed events. Describe context-dependent choices as choices, not universal rules.
3. Give me one editing or sentence-building task using a known case fact. Do not copy the supplied editing example or reveal its answer. Ask me to explain my intended meaning. Stop and wait.
4. After I answer, quote my wording, identify at most two issues, and give a brief explanation and one hint. Ask me to revise before offering a full corrected version. Then accept any accurate, natural alternative that serves the purpose.
5. Continue with two new tasks, one at a time: first guided, then an original response without a sentence frame. End with a two-sentence workplace message using the target feature and a compact self-check. Do not replace this sequence with a worksheet and answer key.

END PROMPT

Copy this prompt online or choose another lesson and support level

Get feedback on a draft

START PROMPT / COPY THROUGH END PROMPT

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Model for comparison AFTER my attempt, not a response to give me first: The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

END REFERENCE

TASK: Coach my workplace writing

1. Ask me to paste my own fictional or anonymized draft and identify its reader and purpose. If the reference gives a writing task, mention that task as the default. If it is a dialogue, suggest a brief follow-up message that records its outcome and open questions. Stop and wait. Do not write the message for me first.
2. Once I supply the draft, check it against the reference. If an ambiguity changes the meaning, ask one focused question before rewriting. Do not assume that the draft is correct evidence for a new deadline, approval, diagnosis, cost or commitment.
3. Give feedback in three parts: one effective phrase with a reason; up to three priority improvements, quoting my words; and one short revision task for me. Prioritize incorrect facts or unclear action before minor grammar. For each language correction, explain why it matters to this reader. Label optional stylistic alternatives separately and preserve my level of certainty and intended politeness.
4. Stop and wait for my revision. Then compare the two attempts, identify an improvement and offer one edited version that preserves my voice and purpose. Keep the requested length; if none is specified, use 70-110 words. Explain any meaningful change rather than silently making the message stronger or more certain.
5. Close with one phrase worth reusing and a short transfer task for a different fictional reader. Wait for my attempt before providing another model.

END PROMPT

Copy this prompt online or choose another lesson and support level